

Providers Under Pressure

Measuring Trans-Affirming Providers' Experiences (TAPE) in the Context of Evolving Healthcare Policies



PRESENTER:

Nat Roberts *(he/they)*

Research, Training and Special Programs Coordinator

Ackerman Institute for the Family's Gender and Family Project

AUTHORS:

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Who We Are

Gender & Family Project's Mission

GFP remains firmly rooted in its mission to empower youth, families and communities by providing gender affirming services, training and research.

We achieve this through the promotion of gender inclusivity as a form of social justice in all the systems involved in the life of the family.

Our Services

- Community groups for caregivers and youth
- Family therapy and clinical advocacy
- Ongoing research studies and involvement in broader TGD research community
- Trainings for schools, clinicians, social workers, caregivers, and employers

FLAIR Lab

Families, Liberation, Affirmation, Intersectionality, and Resilience

Co-founded in 2023 by Nat Roberts, MA and Dr. Dumayi Gutierrez

- **Mission:** Conduct community-centered research to improve the mental health of BIPOC LGBTQIA+ populations.
 - Grounded in minority stress theory, intersectional ecological systems frameworks, queer theory, and anti-colonial frameworks
- **What We Offer:**
 - A nationwide collaborative research team with a dedication in public dissemination of research,
 - Quantitative, qualitative, and mixed-methodology approaches
 - A focus on advancement of systemic change and resilience among BIPOC LGBTQ+ communities.

FLAIR's Work

What We Offer

- A nationwide collaborative research team dedicated to working with oppressed communities and disseminating research in accessible ways
- A focus on advancement of systemic change and resilience among BIPOC LGBTQ+ communities
- Mentorship from a queer / feminist / multicultural lens:
 - Quarterly community meetings
 - Weekly office hours

Active Projects

- Exploring experiences and cultural narratives of Black and Latine sexually and gender expansive (SGE) communities
- Systematic review of clinical and social services for BIPOC SGE clients
- Dev / validation of a modern TGNCNB clinical competency tool
- Continued analysis of TAPE data

Learning Objectives

Participants will...

- **Identify** how structural determinants such as population-based discrimination and hostile legislative environments contribute to provider burnout and workforce instability in gender-affirming and transition-related care. [5.1.1]
- **Assess** how state and federal policies shape the provision of gender-affirming and transition-related care, and apply this knowledge to develop advocacy strategies that protect providers and sustain equitable access to care. [5.2.6]
- **Leverage** their awareness of the harmful outcomes associated with hostile legislative environments to develop effective, non-partisan narratives to influence legislators. [5.2.2]

Subcompetencies

**HESPA II Subcompetencies covered
during this presentation:**

Competency Area 5: Advocacy

- **5.1 – Identify a current or emerging health issue requiring policy, systems, or environmental change.**
 - 5.1.1 – Examine the determinants of health and their underlying causes (e.g., poverty, trauma, and population-based discrimination) related to identified health issues.
- **5.2 – Engage coalitions and stakeholders in addressing the health issue and planning advocacy efforts.**
 - 5.2.2 – Identify factors that influence decision-makers (e.g., societal and cultural norms, financial considerations, upcoming elections, and voting record)
 - 5.2.6 – Identify organizational policies and procedures and federal, state, and local laws that pertain to the advocacy efforts.

Background

GATRC and Anti-Trans Health Legislation

- Gender-affirming / transition-related care (GATRC) is well-documented as a medically necessary and potentially life-saving intervention for trans and gender expansive individuals (Coleman et al., 2022)
- Despite this evidence, legislation which seeks to limit or criminalize the provision of GATRC has grown rapidly in recent years
 - Consequences can include revocation of licensure, criminal or civil liability, or decades in prison
- Harm to patients is well studied and plays a critical role in advocacy
 - Need for research on the harm to providers

Burnout

"... a state of physical, emotional and mental exhaustion that results from long-term involvement in work situations that are emotionally demanding..."

(Schaufeli & Greenglass, 2001, p. 501)

Provider burnout is associated with suboptimal patient care practices (Shanafelt et al., 2002) and poorer patient safety (Hall et al., 2016).

Developing Measuring TAPE

- FLAIR Lab's inaugural project
- Developed on "QT Research Collaborative" calls
- Shared interest in studying the impact of legislation on providers as an advocacy strategy
 - *How can we investigate the relationship between anti-trans health legislation and stress on healthcare systems overall?*
 - *How can we leverage research to build broader coalitions to defend GATRC?*

Measuring TAPE

GOAL: Measuring Trans-Affirming Providers' Experiences

- A mixed-methods study assessing the experiences of providers of GATRC in the current context of anti-trans legislation
- Cross-sectional survey – provider's demographics, orientation of care, questions exploring the impact of anti-trans legislation on their personal and professional lives, and validated measures
 - *Preliminary qualitative results will be presented at 2026 LGBT Workforce Conference in NYC next week!*

Team (alphabetically):

Shannon Collins,
Dumayi Gutierrez,
Stella Jendrzewski,
Nüma Keesha Molière,
Andy Price,
Nat Roberts (PI),
Theo Stein,
Kalen D. Zeiger

- **Launched:** Sep 1, 2024
- **Closed:** Mar 31, 2025

Methods

Demographics

Demographics collected:

- Age
- Race/Ethnicity
- Self-identification of gender & trans identity
- Sexuality
- Household income
- Occupation (behavioral, medical, or both)
- Licensure
- States licensed
- Medical specialty
- Work with minors (Y/N)
- Work predominantly with trans clients (Y/N)
- Work setting
- Frequency of telehealth

Measures

Copenhagen Burnout Inventory (Kristensen et al., 2005)

- **Personal** (e.g., How often do you feel worn out?)
- **Work** (e.g., Are you exhausted at the thought of another day at work?)
- **Client** (e.g., Does it drain your energy to work with clients?)

LGBTQ Resilience & Inequities (Murray & Dailey, 2020)

- **Resilience** (e.g., I feel a sense of shared hardship with the broader LGBTQ+ community.)
- **Inequities** (e.g., My identities put me at a disadvantage within the LGBTQ+ community).

Multidimensional Scale of Perceived Social Support (Zimmet et al., 1988)

- **Friends** (e.g., I can talk about my problems with my friends.)
- **Family** (e.g., My family really tries to help me.)
- **Significant Other** (e.g., I have a special person who is a real source of comfort to me.)
- **Overall** score averages the 3 domains

Brief Resilience Scale (Smith et al., 2008)

- (e.g., I tend to take a long time to get over setbacks in my life.)

Our Questions

Experiences

- Received threats (via workplace address, home address, by email, by phone)
- Harassed at work or in public
- Experienced physical violence
- Pushback from employer regarding provision of GATRC
- Experienced negative impact on liability insurance
- Experienced licensing board disciplinary action; fined
- Charged with child abuse
- Sued; faced criminal charges; faced jail time
- Removed contact info from internet
- Sought out legal services
- Took down pride flags
- Forced to delay patient care; refer out
- Considered moving out of state / country

Attitudes

- I feel afraid.
- I feel motivated.
- I feel my physical safety is at risk.
- I am worried about losing my job.
- I feel that remote/virtual work is safer than in-person work.
- I am more determined to provide GATRC.
- I no longer want to provide GATRC.

Sample and Recruitment

Inclusion criteria

- Licensed medical or mental health practitioner
- Practiced in the United States within the past 4 years
- Provided gender-affirming or transition related care (GATRC) to at least one patient in the past 4 years

Recruitment

- Emails and cold calls to clinics and LGBT centers via CenterLink Directory
- Registered with Planned Parenthood to circulate internally
- Shared in Trans PATH newsletter
- Shared with GLMA members
- Shared within professional networks (e.g., therapist listservs)

Data Cleaning

- Received 133 responses
- 8 submissions removed
 - *5 did not confirm that they were a provider of GATRC*
 - *2 removed because they did not complete the survey*
 - *1 duplicate submission*
- Final N = 124

Data Cleaning (cont.)

Data wrangling completed in R

- Renaming columns, cleaning text, recoding and collapsing factors (e.g., sexuality, gender identity, income)
- One-hot-encoding race/ethnicity and state of licensure
- Calculating scale totals and means
- Calculating within participant variance as bot detection strategy
 - (Betts et al., 2024)
- Creating functions to assign highest and lowest policy score to participants licensed in multiple states and calculate difference scores

Analyses

- Demographics
 - Descriptive statistics (mean, SD) for validated scales
 - Prevalence (%) of experiences and attitudes for overall sample and subgroups
- T-tests for validated scales to compare subgroups
 - Using `t.test()` in R (two-tailed)
 - Bonferroni adjustment for critical p value ($p = 0.007$)
- Risk ratios calculated for experiences among subsamples
 - Race, trans identity, sexuality, providing care to minors, providing care predominantly to trans clients, use of telehealth, and household income

Analyses

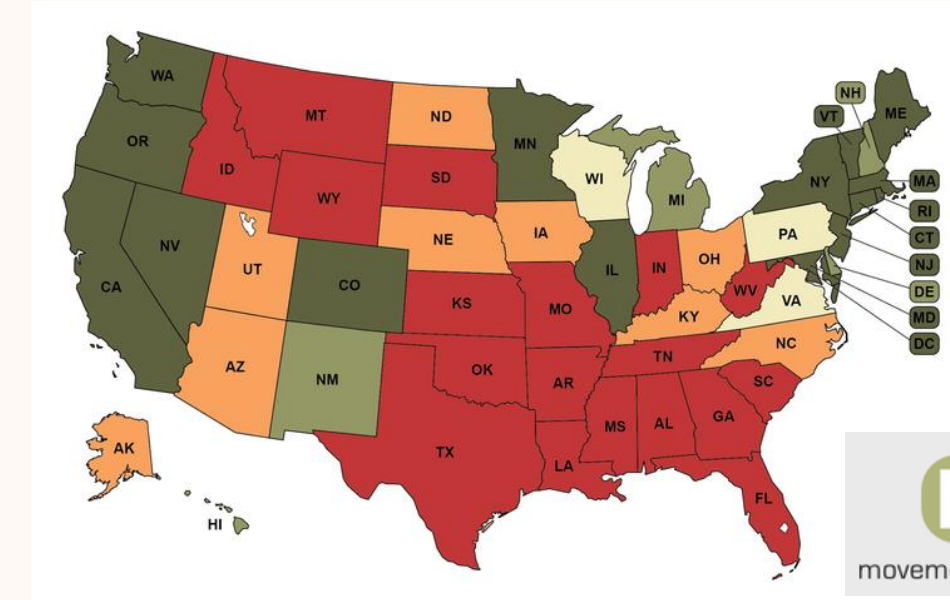
Testing Provider Burnout ~ Policy Environment

Two methods

- Cunningham et al (2022)
 - Tested policy ~ depression / suicide searches on Google in (Jul 2019 - Jul 2020)
 - Our data was too sparse
- Dubois et al (2018)
 - Tested policy ~ trans health outcomes (physical health, mental health, health behaviors, healthcare utilization)
 - Used Movement Advancement Project's LGBT Policy Tally

Our method

- Created MAP Policy Tally reference file
 - Overall LGBT policy score
 - LGBT healthcare score
 - Gender identity related policy score
- Updated 11/5/2024
 - Provided accurate policy scores for all of data collection, with a few exceptions
 - PA: Passed resolution recognizing Trans Awareness Week instead of TDOR
 - MA: Passed act regarding chest baring equality
 - NJ + NY: Bills considered but not passed



Hypothesis & Models

Hypothesis:

Burnout can be predicted by the policy environments in which providers are licensed.

MODEL 1: Burnout is predicted by the most restrictive policy environment in which a provider is licensed.

$$Y_i = \beta_0 + \beta_1(\text{Lowest GIPT}_i) + \varepsilon_i$$

MODEL 2: Burnout is predicted by the most protective policy environment in which a provider is licensed.

$$Y_i = \beta_0 + \beta_1(\text{Highest GIPT}_i) + \varepsilon_i$$

MODEL 3: Burnout is predicted by the most restrictive policy environment and the disparity between policy environments, while accounting for providers who do not hold multiple licenses.

$$Y_i = \beta_0 + \beta_1(\text{Lowest GIPT}_i) + \beta_2(\text{GIPT Difference}_i) + \beta_3(\text{Multiple Licensure}_i) + \varepsilon_i$$

Results: Descriptives

Results: Sample Demographics

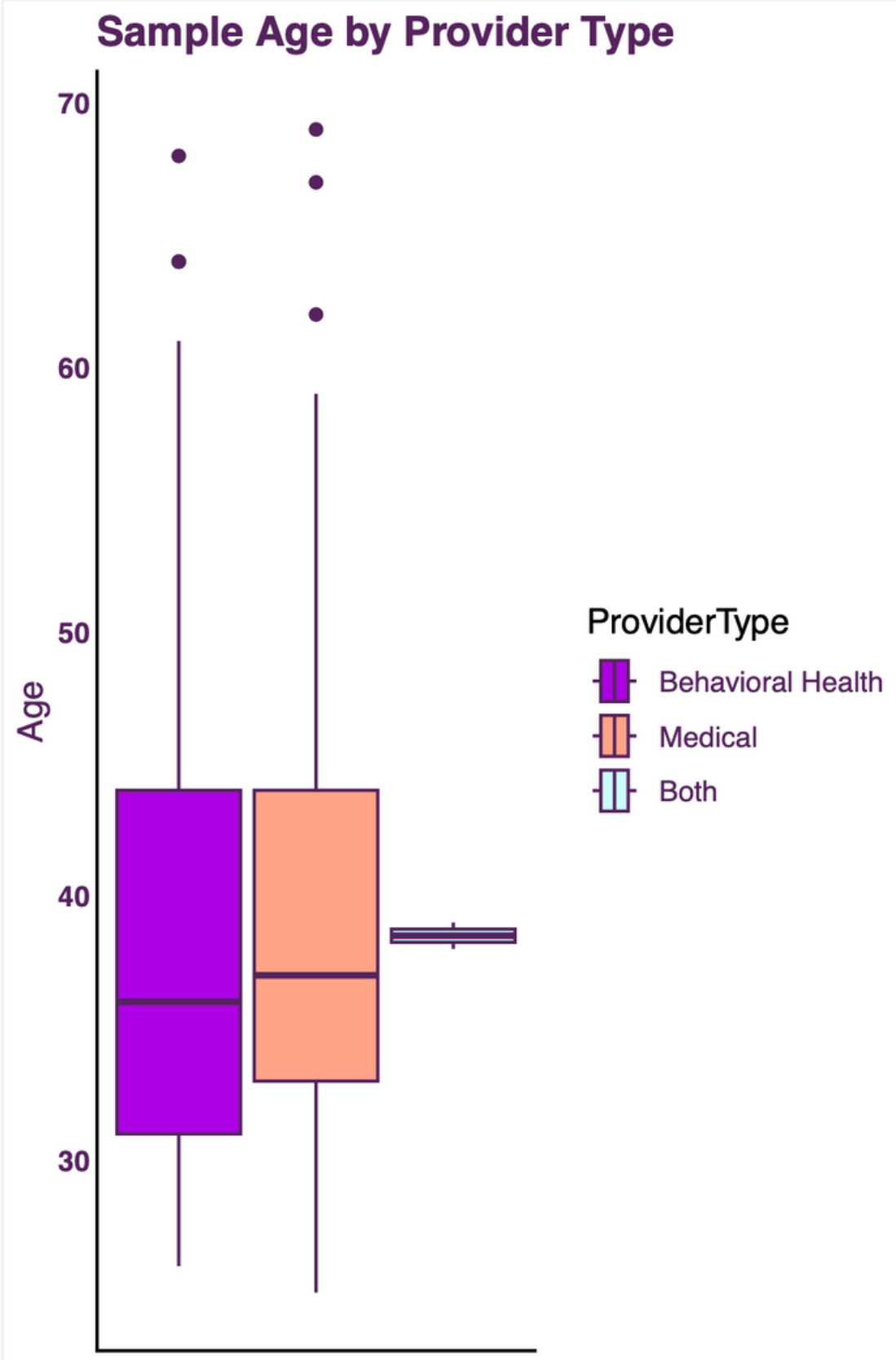


Table 1

Sociodemographic Characteristics of Participants

Demographic		
	n	%
Race		
white	106	85.5
Not white	18	14.5
Trans		
Yes	35	28.2
No	89	71.8
Sexuality		
Not Straight	92	74.2
Straight	32	25.8
Intersex		
Yes	123	99.2
No	1	0.81
Occupation		
Medical Provider	52	41.9
Behavioral Provider	69	55.6
Medical + Behavioral	3	2.42
Work with minors		
Yes	88	71.0
No	36	29.0
Primarily Trans Clients		
Yes	74	59.7
No	50	40.3
Telehealth		
Frequent	78	62.9
Not Frequent	46	37.1
Income		
<100k	40	32.3
=>100k	84	67.7

Note. N = 124. Participants were on average 39.22 years old (SD = 10.21)

Results: Sample Demographics

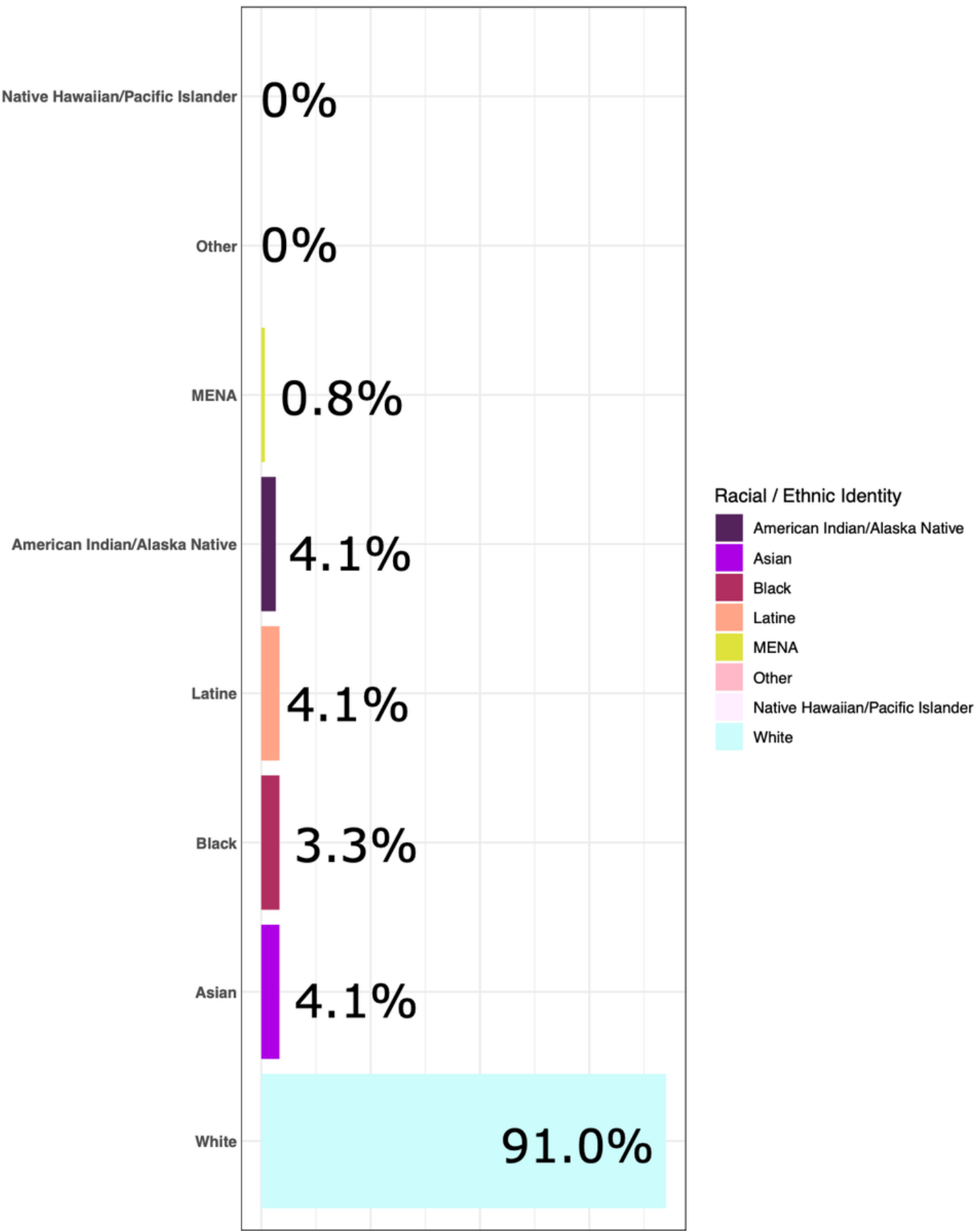
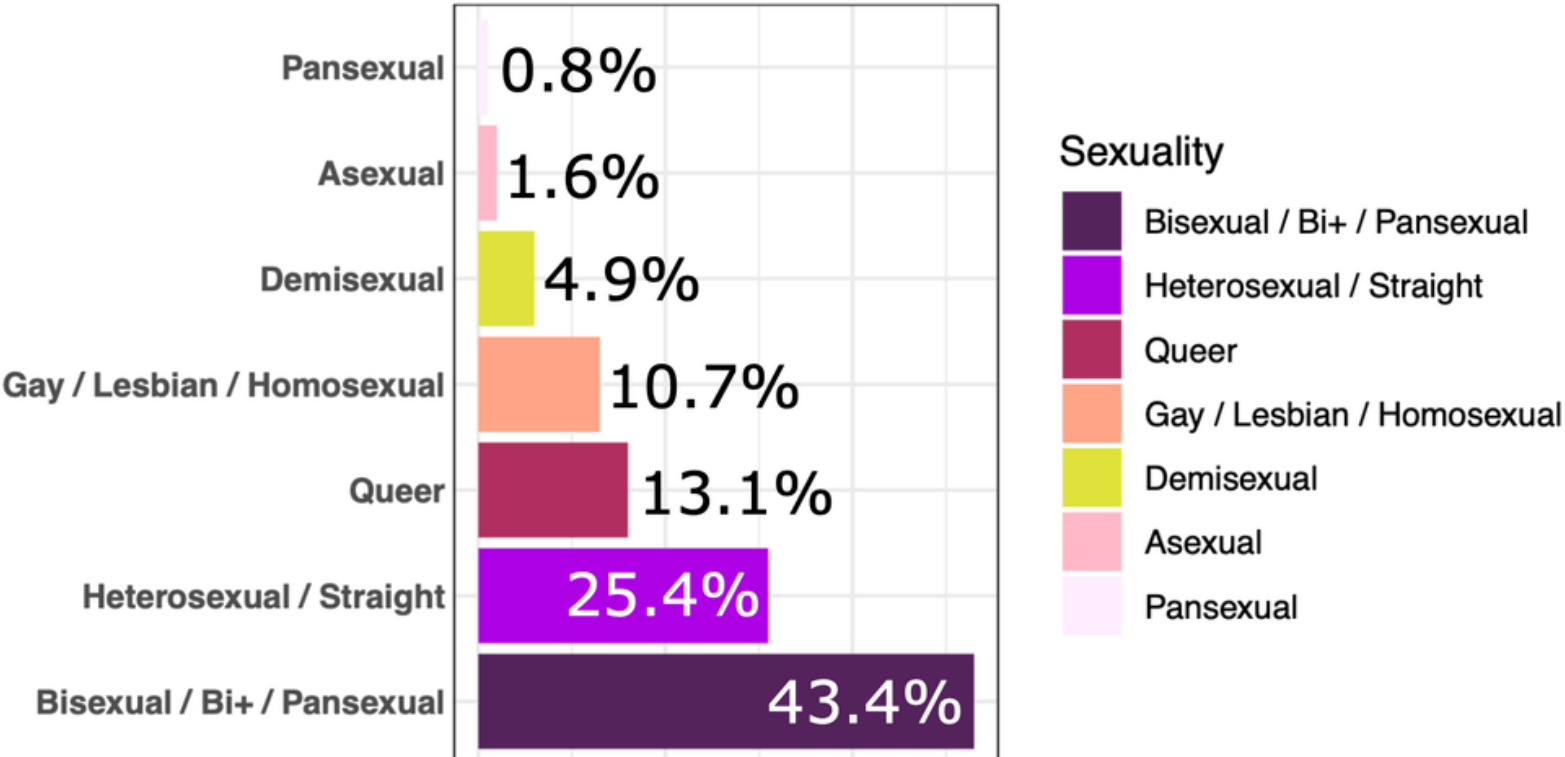
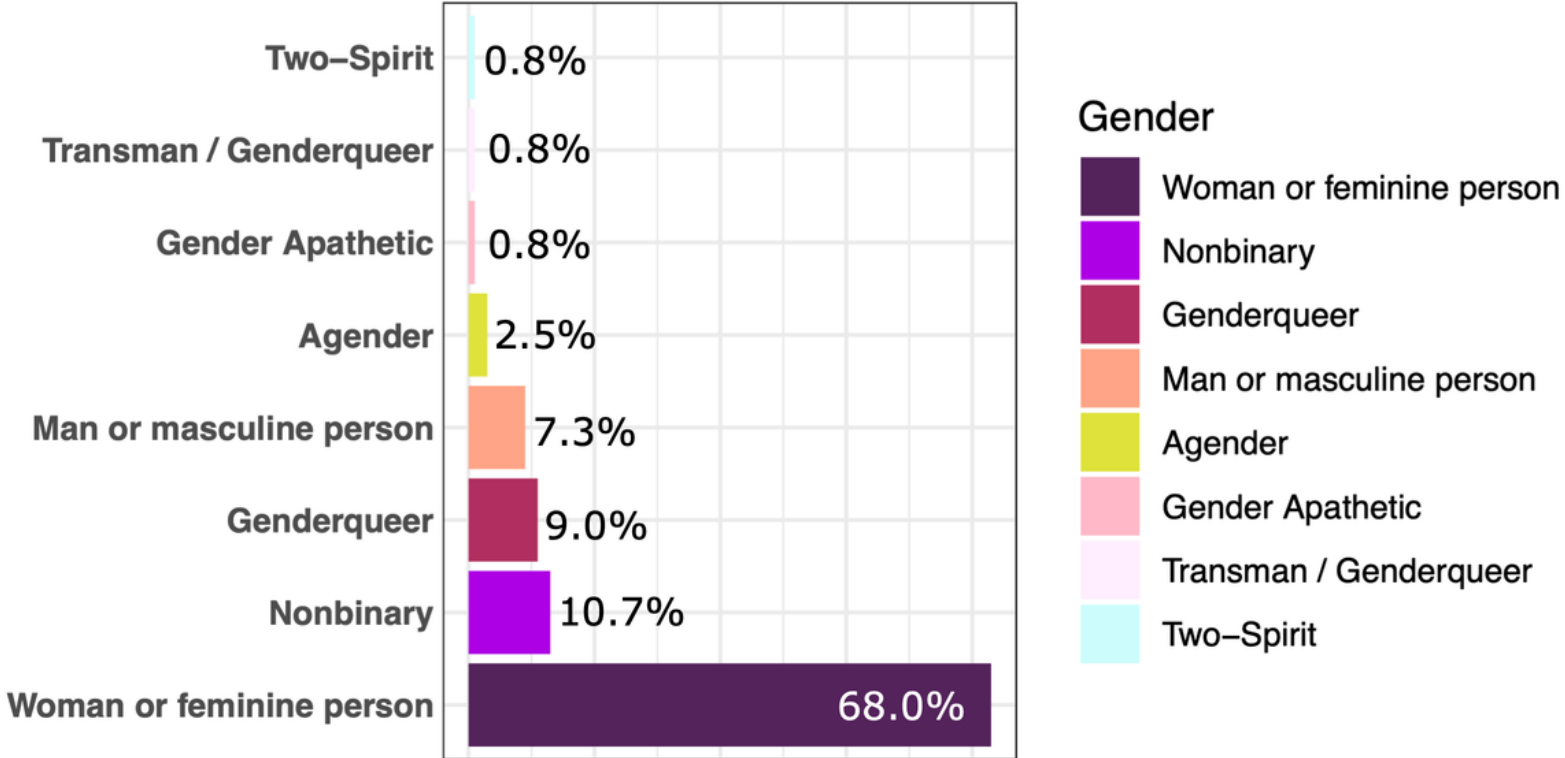


Table of Overall Prevalence and Risk Ratios for Subgroups

	Full Sample (%)	Non-White	Trans	Queer	Serves Minors	Predom. Trans Clients	Infrequent Telehealth Use	Income < \$100k
Threats at work	16.9	2.35	1.90	0.69	3.88	2.16	0.53	0.84
Threats by email	12.1	0.90	2.22	2.26	0.82	1.35	0.62	1.40
Threats by phone	7.26	1.68	2.03	0.69	0.51	2.37	0.22	1.05
Harass at work	21	1.76	2.10	1.15	0.92	1.84	0.75	1.54
Harass in public	28	1.81	3.78	1.34	1.33	2.19	0.44	1.47
Physical violence	3.2	5.87	7.39	1.04	1.23	no data	no data	6.30
Work pushback	30.6	1.33	1.39	1.85	1.54	1.46	0.88	1.53
Insurance impact	4.03	no data	3.68	1.39	1.64	no data	0.42	3.15
Sued	0.81	no data	no data	no data	no data	no data	no data	no data
Removed info from web	41.9	1.58	1.30	0.78	1.72	1.17	0.75	0.63
Sought legal services	15.3	1.11	0.87	1.30	1.53	2.54	1.00	0.56
Removed affirming signs	11.3	0.98	5.53	2.08	1.03	1.69	1.26	5.67
Delayed patient care	23.4	1.23	1.23	0.77	1.08	1.29	0.54	0.67
Considered moving	29.8	0.92	1.87	2.87	1.49	1.83	0.72	2.00

Relative Risk

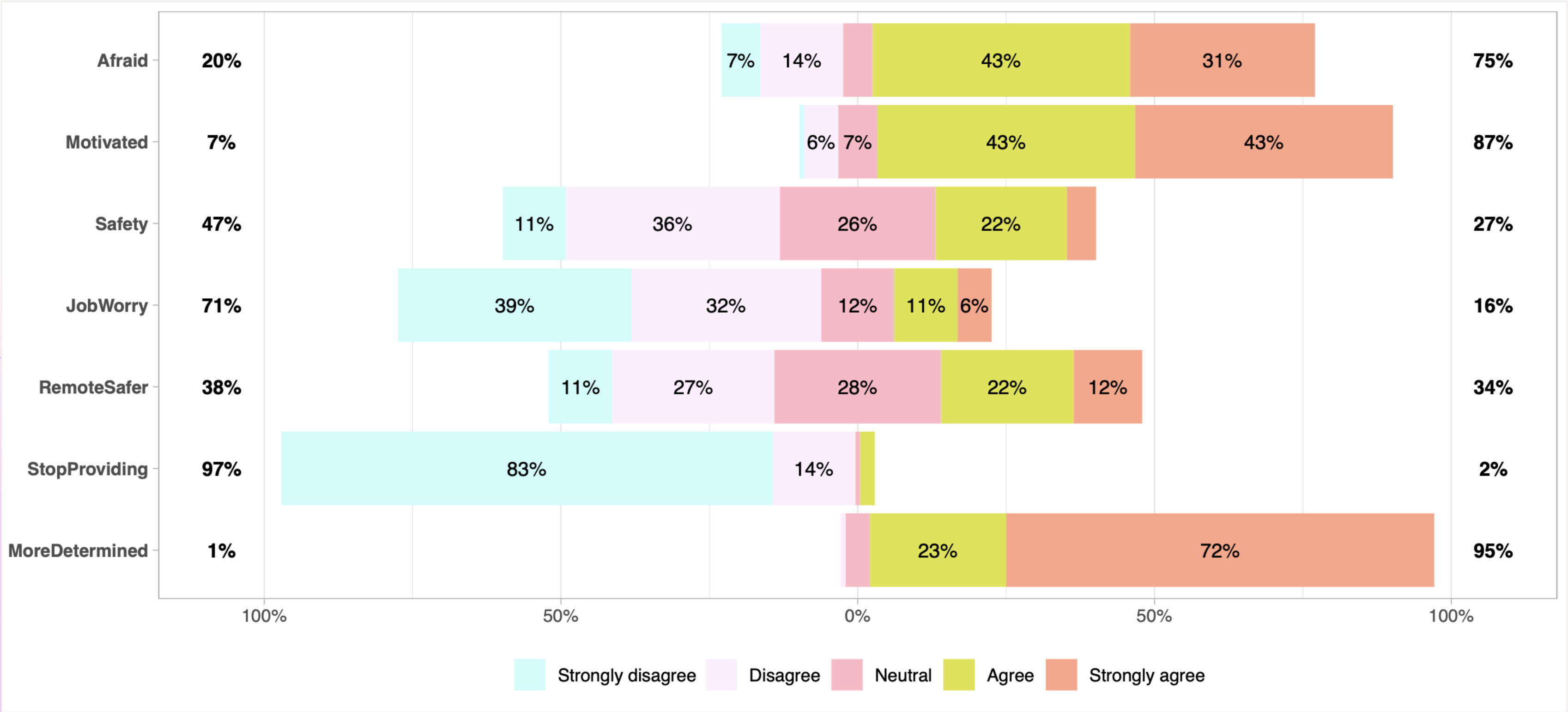
2.0 - 3.4

3.5 - 4.9

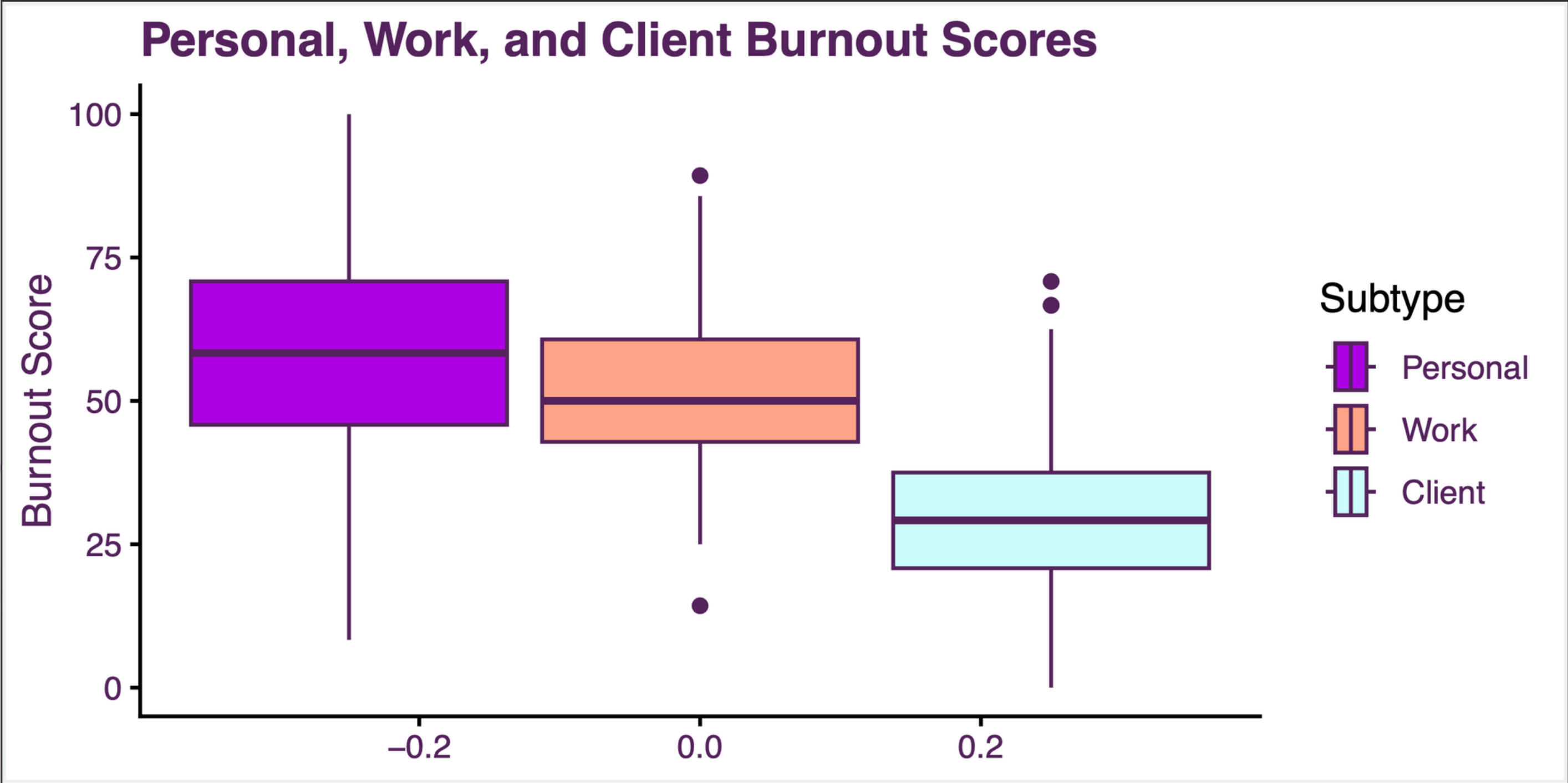
5.0+

No subjects reported threats at home, licensing disciplinary action, reported for child abuse, faced criminal charges, faced jail time, fined

Results: Attitudes

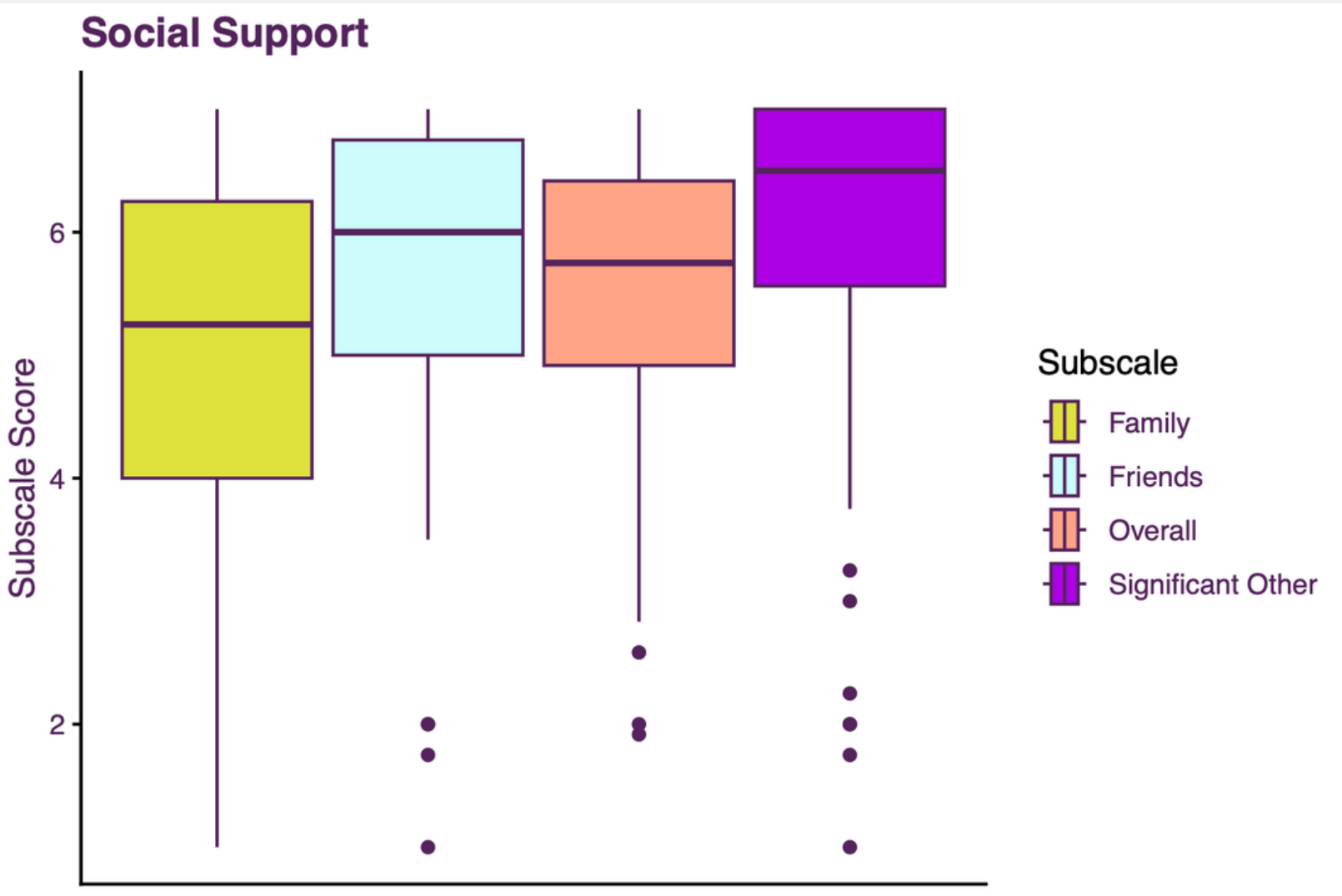


Results: Scale Means & Distributions



Copenhagen Burnout Inventory scoring:
50-74 = Moderate; 75-99 = High; 100 = Severe

Results: Scale Means & Distributions



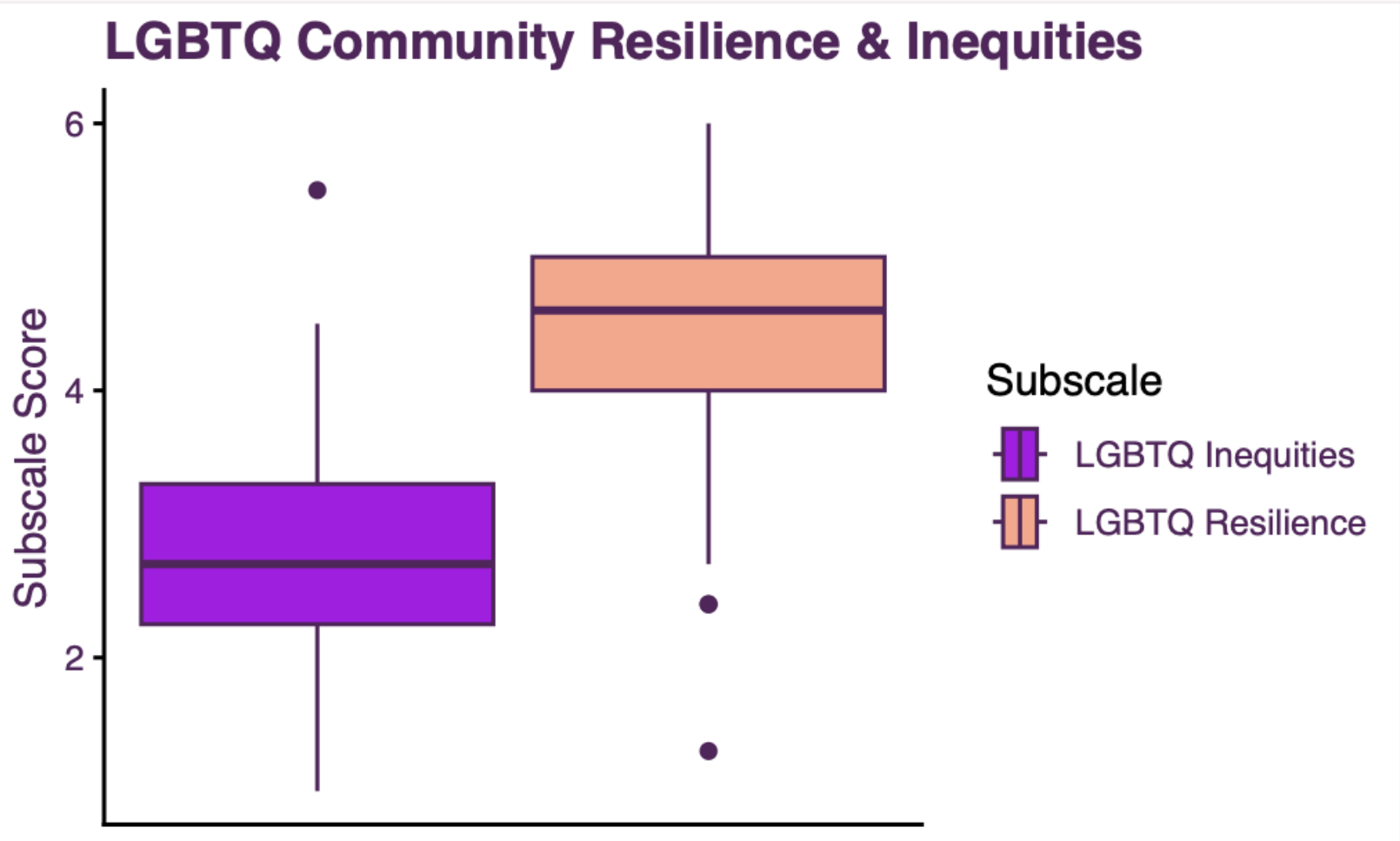
MSPSS scoring:

1-2.9 = Low social support
3-5 = Moderate social support
5.1-7 = High social support



Brief Resilience Scale scoring:

1-2.3 = Low resilience
2.4-3.6 = Moderate resilience
3.7-5.0 = High resilience

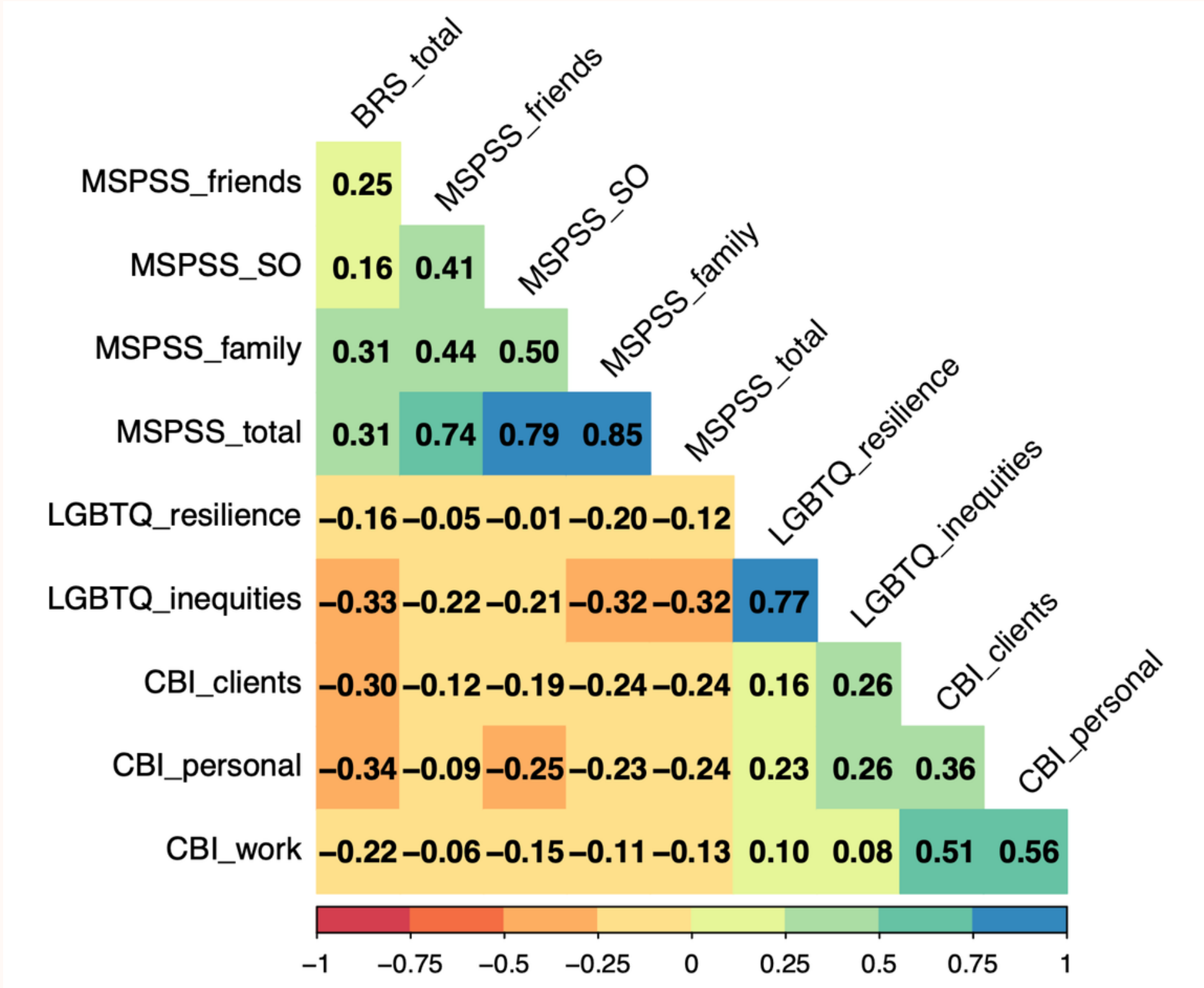


LGBTQ Resilience & Inequities scoring::

Subscale items averaged. No suggested scoring system

Results: Correlations

- **Work Burnout**
 - **Negative:** Resilience
- **Personal Burnout**
 - **Negative:** Resilience, social support (SO, family, overall)
 - **Positive:** LGBTQ resilience, LGBTQ inequities
- **Client Burnout**
 - **Negative:** Resilience
 - **Positive:** LGBTQ inequities



Results: Testing Models

Running Linear Models

We tested all models with:

- DV = 3 subtypes of Burnout (Personal, Work, Client)
- IV = Gender Identity Policy Tally (GIPT) score

Models tested on Work and Client Burnout explained very little variance:

- P values ranged from 0.12 (M3 Client, mutliL) to 0.82 (M3 Work, multiL)
- R2 values ranged from 0.0002 (M2 Client) to 0.030 (M3 Client)

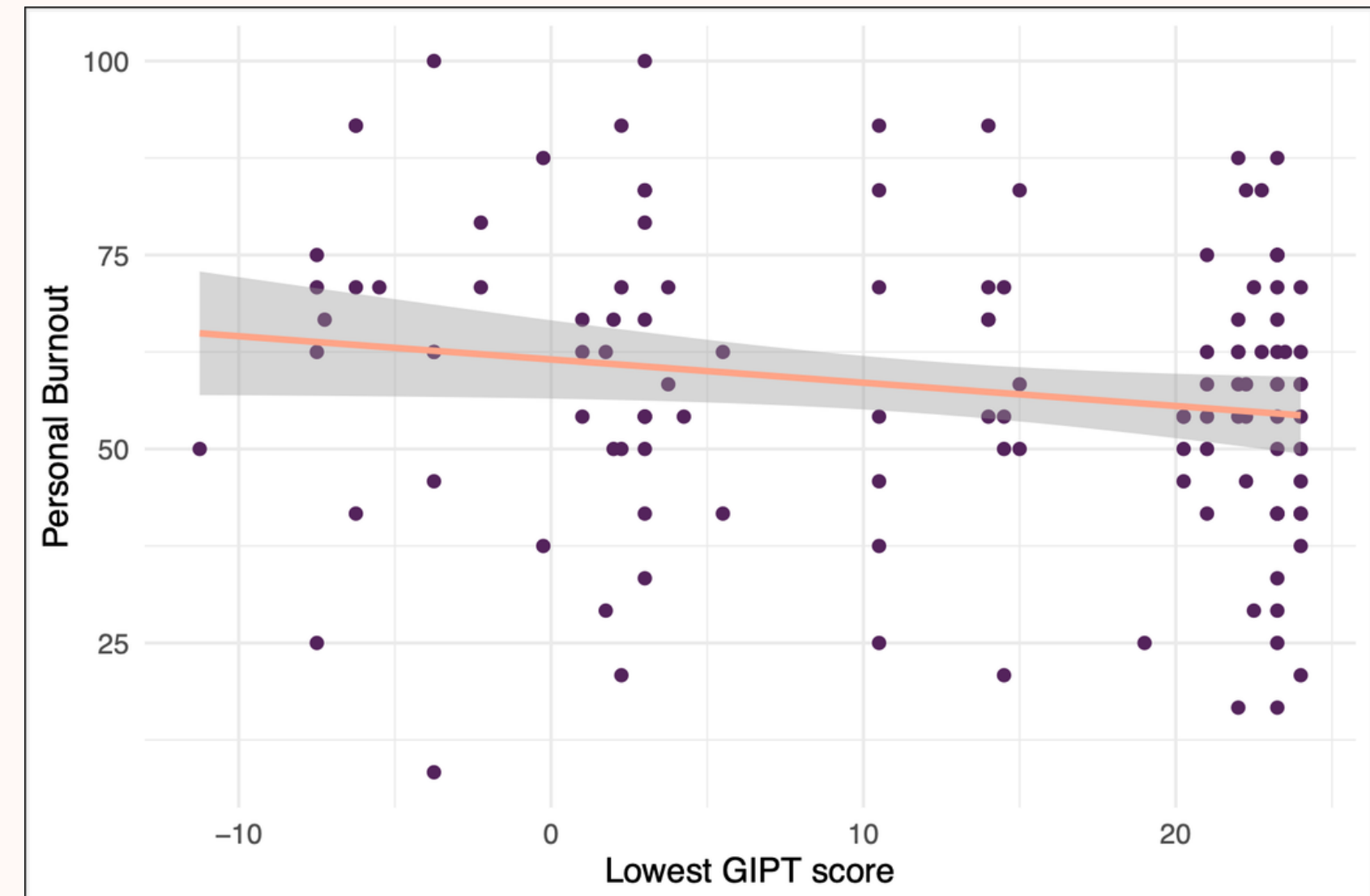
Presented results will focus on Personal Burnout, which:

- Approached significance in some models
- Had the greatest variance and highest mean
- Larger correlations with more covariates
 - $r > +/- 0.25$ for resilience, support from significant other, LGBTQ inequities

Model #1

$$Y_i = \beta_0 + \beta_1(\text{Lowest GIPT}_i) + \varepsilon_i$$

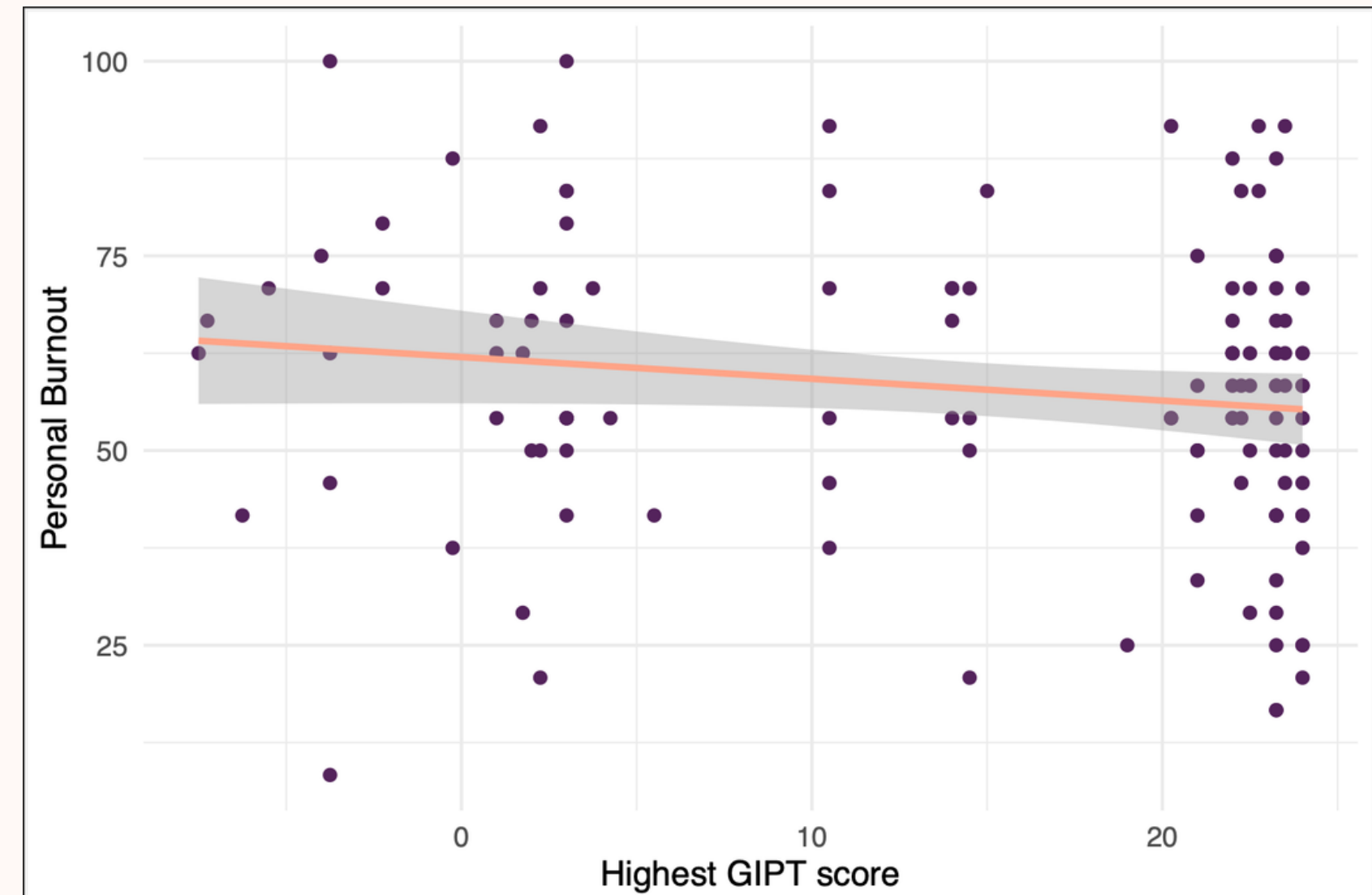
```
## Call:
## lm(formula = CBI_personal ~ total_L0, data = tapedata)
##
## Residuals:
##      Min       1Q   Median       3Q      Max
## -54.327 -10.918   0.552  11.017  39.363
##
## Coefficients:
##              Estimate Std. Error t value Pr(>|t|)
## (Intercept)  61.5362     2.5498  24.134  <2e-16 ***
## total_L0     -0.2997     0.1554  -1.929   0.0561 .
## ---
## Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
##
## Residual standard error: 18.9 on 120 degrees of freedom
## Multiple R-squared:  0.03008,    Adjusted R-squared:  0.022
## F-statistic: 3.722 on 1 and 120 DF,  p-value: 0.05607
```



Model #2

$$Y_i = \beta_0 + \beta_1(\text{Highest GIPT}_i) + \varepsilon_i$$

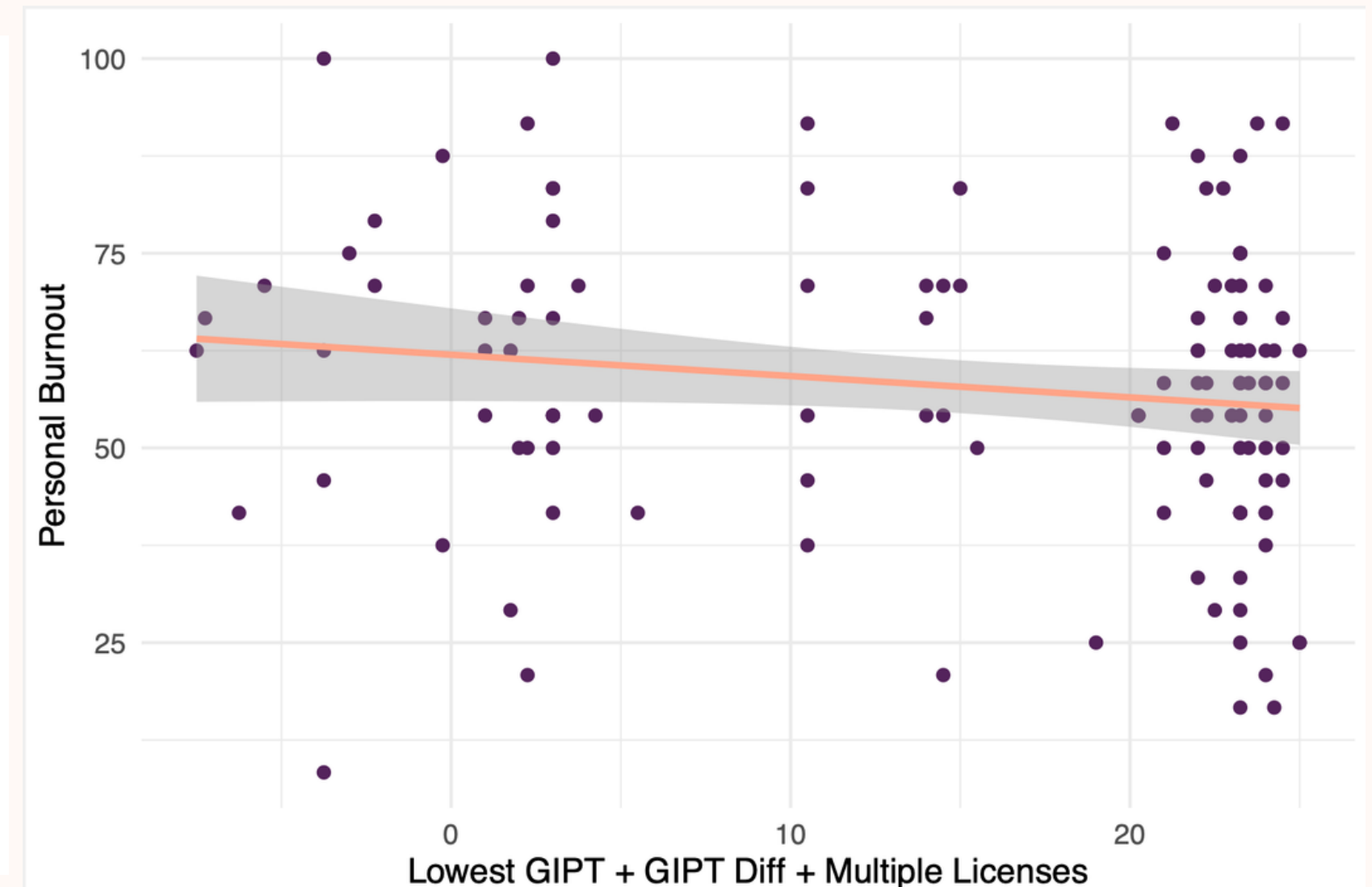
```
## Call:
## lm(formula = CBI_personal ~ total_HI, data = tapedata)
##
## Residuals:
##      Min       1Q   Median       3Q      Max
## -54.724 -11.327   1.589  11.628  38.830
##
## Coefficients:
##              Estimate Std. Error t value Pr(>|t|)
## (Intercept)  62.0088     3.0077  20.617  <2e-16 ***
## total_HI     -0.2796     0.1676  -1.668   0.0978 .
## ---
## Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
##
## Residual standard error: 18.97 on 120 degrees of freedom
## Multiple R-squared:  0.02267,    Adjusted R-squared:  0.01453
## F-statistic: 2.784 on 1 and 120 DF,  p-value: 0.09783
```



Model #3

$$Y_i = \beta_0 + \beta_1(\text{Lowest GIPT}_i) + \beta_2(\text{GIPT Difference}_i) + \beta_3(\text{Multiple Licensure}_i) + \varepsilon_i$$

```
## Call:
## lm(formula = CBI_personal ~ total_L0 + total_DIFF + multiL, data = tapedata)
##
## Residuals:
##      Min       1Q   Median       3Q      Max
## -55.138 -11.608   0.736  11.414  38.625
##
## Coefficients:
##              Estimate Std. Error t value Pr(>|t|)
## (Intercept)  62.30691    3.02848  20.574  <2e-16 ***
## total_L0     -0.31052    0.17546  -1.770   0.0794 .
## total_DIFF    0.06193    0.40465   0.153   0.8786
## multiL       -3.29577    6.78313  -0.486   0.6280
## ---
## Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
##
## Residual standard error: 19.02 on 117 degrees of freedom
## (1 observation deleted due to missingness)
## Multiple R-squared:  0.03362,    Adjusted R-squared:  0.008837
## F-statistic: 1.357 on 3 and 117 DF,  p-value: 0.2596
```

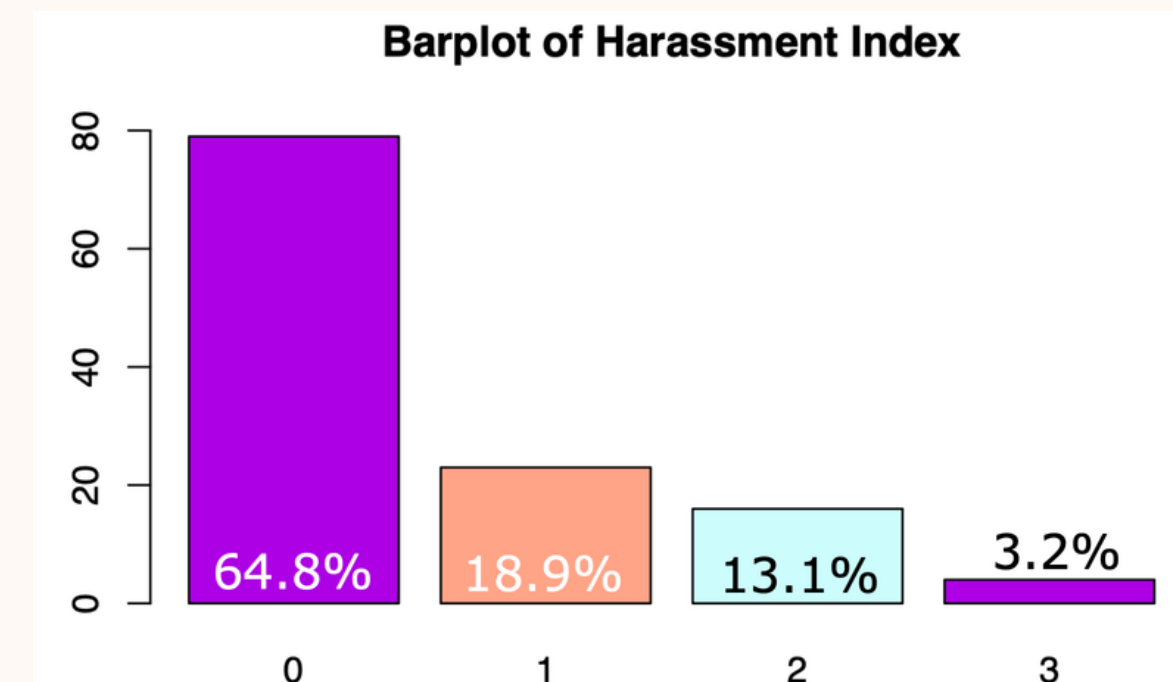
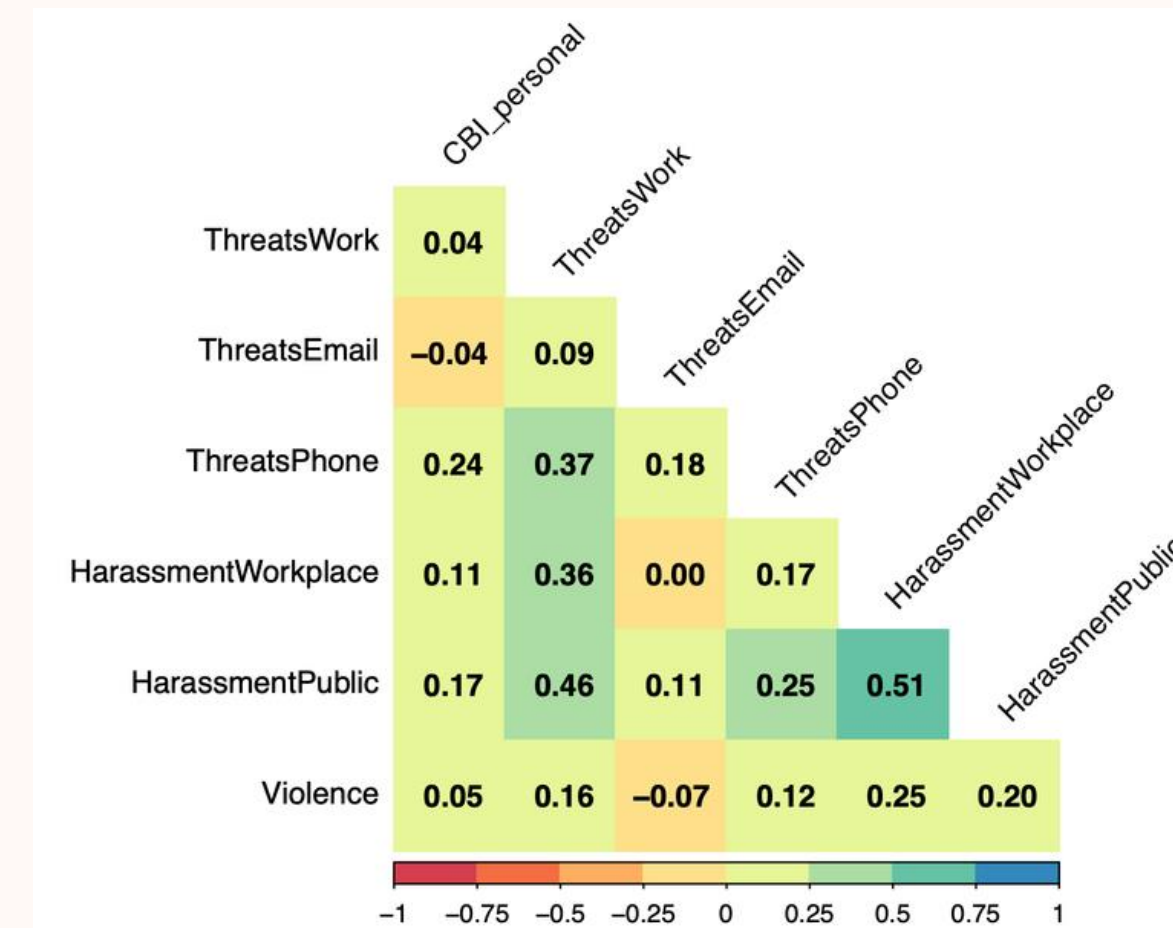


Results: Post Hoc Analyses

Post Hoc Analyses

Explored negative experiences as predictor

- Focused on experiences with strongest correlations to Personal Burnout
 - Phone Threats ($r \approx .24$)
 - Harassment in Public ($r \approx .17$)
 - Harassment at Work ($r \approx .11$)
- Created Harassment Index
- Also considered significant identity-related factors
 - Queer identity
 - Trans identity



Post Hoc Analyses

Harassment Index predicted
Personal Burnout:

```
## Call:
## lm(formula = CBI_personal ~ harassmt_index, data = tapedata)
##
## Residuals:
##      Min       1Q   Median       3Q      Max
## -46.845 -12.855   0.687  11.489  39.885
##
## Coefficients:
##              Estimate Std. Error t value Pr(>|t|)
## (Intercept)      55.178     2.025  27.242  <2e-16 ***
## harassmt_index    4.937     2.017   2.447  0.0158 *
## ---
## Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
##
## Residual standard error: 18.73 on 120 degrees of freedom
## Multiple R-squared:  0.04754, Adjusted R-squared:  0.0396
## F-statistic: 5.99 on 1 and 120 DF, p-value: 0.01584
```

Also explored
queer and
trans identity
as interaction
terms

```
## Call:
## lm(formula = CBI_personal ~ harassmt_index * sexuality_collapse,
##      data = tapedata)
##
## Residuals:
##      Min       1Q   Median       3Q      Max
## -50.094  -8.401   0.010   8.344  41.917
##
## Coefficients:
##              Estimate Std. Error t value Pr(>|t|)
## (Intercept)      58.427     2.284  25.587  <2e-16 ***
## harassmt_index    5.143     2.309   2.227  0.0278 *
## sexuality_collapsestraight -12.604     4.352  -2.896  0.0045 **
## harassmt_index:sexuality_collapsestraight -1.216     4.224  -0.288  0.7739
## ---
## Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
##
## Residual standard error: 17.95 on 118 degrees of freedom
## Multiple R-squared:  0.1399, Adjusted R-squared:  0.1181
## F-statistic: 6.399 on 3 and 118 DF, p-value: 0.0004698
```

```
## Call:
## lm(formula = CBI_personal ~ harassmt_index * trans_collapse,
##      data = tapedata)
##
## Residuals:
##      Min       1Q   Median       3Q      Max
## -47.448  -9.903   0.911  11.157  45.397
##
## Coefficients:
##              Estimate Std. Error t value Pr(>|t|)
## (Intercept)      53.256     2.163  24.619  <2e-16 ***
## harassmt_index    1.347     2.721   0.495  0.6215
## trans_collapseYes 10.859     5.009   2.168  0.0322 *
## harassmt_index:trans_collapseYes 2.639     4.232   0.624  0.5340
## ---
## Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
##
## Residual standard error: 18.04 on 118 degrees of freedom
## Multiple R-squared:  0.1313, Adjusted R-squared:  0.1092
## F-statistic: 5.947 on 3 and 118 DF, p-value: 0.0008214
```

Key Insights

Burnout, Policy & Harassment

- Personal Burnout is significantly higher among
 - Trans providers ($p < 0.001$)
 - Queer providers ($p < 0.001$)
 - Provider with income $< \$100k$ ($p < 0.001$)
- Policy Analysis
 - Models were not statistically significant
 - Personal Burnout ~ GIPT_LO approached significance ($p = 0.0561$)
 - Insufficient power to detect significant effect
 - Also suggests providers more impacted by overall political environment, rather than state-specific bills
- Key harassment experiences predict burnout
 - No significant interaction with queer or trans identity, though significant alone
 - Suggests harassment is not segmented by provider SGM identity
 - However, burnout variance better explained by harassment + identity

Limitations

Limitations

- Cross-sectional design
 - Causation
 - History
- Sample
 - Convenience & snowball sampling
 - Predominantly white (85.5%) – may not adequately represent the experiences of racially diverse TGE providers
 - 13 States not represented: AL, AR, DE, GA, HI, KS, KY, MS, ND, OK, SC, SD, WY
 - No representation from U.S. territories (Puerto Rico, Guam, American Samoa, U.S. Virgin Islands)

Advocating for Providers and GATRC

Advocating for Providers and GATRC

Though our results did not find a significant association between policy scores and provider burnout, the harm of the overall climate is evident in:

- Personal & work burnout (over 50% of sample in Moderate range)
- Reported experiences of harassment
- Reported feelings of fear and job worry

Advocating for Providers and GATRC

Results also suggest that provider well-being is impacted by:

- Institutional harassment
 - E.g., conservative media targeting academic hospitals
- Workplace stressors
 - E.g., transphobic colleagues, institutions choosing to “comply in advance”
- Secondary trauma experienced by serving clients in distress
 - E.g., powerlessness, hopelessness, emotional exhaustion
- Identity-related factors and socioeconomic status
 - Significantly higher burnout among providers who are trans, queer, or earning <100k

Framing the Problem

***How might you advocate for providers, patients or
GATRC with what you learned today?***

Suggestions (micro to macro):

- Nearly 42% of providers removed their information from the web as a safety strategy.
 - **ACTION:** Institutions should provide data removal services to providers and involve providers in relevant conversations about security, data management, and emergency planning.
- Nearly 30% of providers have considering moving out of state or out of the country in response to anti-trans healthcare legislation.
 - **ACTION:** Professional organizations and provider-led coalitions can lobby against legislation on the state level by emphasizing the potential for “brain drain”

Suggestions (micro to macro):

- Personal and work burnout in moderate range for > 50% of sample
 - **ACTION:** All providers need support right now, especially trans, queer and lower income; institutions should avoid adding to burnout, create supports, and involve providers in decisions
- Provider burnout is most directly associated with experiences of harassment
 - **ACTION:** Groups challenging federal-level legislation should emphasize the physical danger and psychological harm that providers face as a result of political rhetoric and misinformation.
 - Lenning et al (2021): Violent ideology → Violent policy → Violent action

Future Directions

Future Directions

Where are providers now?

- We captured a critical time period, but legislative change is constant
- How do other sociopolitical stressors (e.g., mass deportations, police violence) negatively impact providers and health care systems serving oppressed communities?

Suggestions for future studies on GATRC provider well-being

- Critical: Ensuring privacy and safety of providers
- Exploring alternative approaches to measuring legislative climate (Cunningham et al., 2022)
- Longitudinal and biobehavioral approaches (tracking cortisol, using surveys or wellness apps to observe ongoing stress)
- Qualitative approaches (interviews and focus groups, PAR to develop recommendations for employers)
- Continue to draw connections between provider well-being and sustainable healthcare systems – we are not well when our providers are not well!

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Questions?

Thank You!
